COVID-19 POLICY

SUBJECT COVID-19

DATE November 2021

SANITARY PROCEDURES AND PRACTICES

POLICY

In addition to routine practices for dealing with blood/body fluids and blood/body fluids by-products, all staff must strictly adhere to sanitary precautions in all aspects of the program.

PROCEDURES

Refer to Public Health Ontario, Health Department Wee Care Manual, Public Health Ontario's Environmental Cleaning and Disinfection fact sheet, the Public Services Health and Safety Association's Child Care Centre Employer Guideline, and the Reopening Tool Kit for information and best practices for cleaning and disinfecting.

Child Care Premises will be cleaned thoroughly prior to re-opening and daily on an ongoing basis. In addition, frequently touched surfaces should be cleaned and disinfected at least twice daily as a suggested at a minimum. However, more frequent cleaning and disinfection may be necessary, depending on the frequency of use and extent of soilage. Cleaning and disinfecting should be logged to track and demonstrate cleaning schedules.

Cleaning

- Use detergent and warm water to clean visibly soiled surfaces
- Rinse the surface with clean water (warm to tepid temperature preferred) to ensure detergent is removed
- Let the surface dry

Disinfectant/Sanitizer

The use of an approved disinfecting product from the List of hard-surface disinfectants and hand sanitizer COVID-19 is required.

Clean and disinfect upon ENTRY to child care (for staff):

- Any hard surfaces such as water bottles, travel mugs, cell phones, lunch containers

Clean and disinfect upon children's ENTRY to child care:

- Any hard surfaces such as water bottles, containers

Disposable Gloves

Gloves must be worn in the following circumstances and disposed after use:

- Administering first aid
- Cleaning and disinfecting blood and body fluid (e.g. vomitus spills)
- Rinsing wet clothing or linen
- Contact with diarrhea including cleaning and disinfecting contaminated surfaces, diaper changing
- During active screening and in the case of an isolation of an ill child

Medical Masks and Eye Protection

- All staff, care providers and students completing post-secondary placements are required to consistently wear medical masks while inside the premesis, (unless eating - but time with masks off should be limited and physical distancing should be maintained).
- A medical mask must be worn at all times by anyone who has received their COVID-19 vaccination in the past 48 hours and are experiencing what may be post-vaccine symptoms (i.e., mild headache, fatigue, muscle aches, and/or joint pain) that only began after vaccination.
- Medical masks are required to be worn outdoors by all staff and students completing a post-secondary placement if two meters of physical distance from others cannot be maintained.
- Eye protection (i.e., face shields or goggles) is required for all staff and students completing post-secondary placements if they come within two metres of an unmasked individual both indoors and outdoors.
- All children in grades one and above are required to wear a non-medical mask or face covering while inside the premises. Masks or face coverings may be removed for the purposes of eating, drinking, and sleeping.
- Children are not required to wear masks outdoors.
- Parents/guardians are responsible for providing their children with a mask and should be reminded that their child will require a way to store their mask when not in use.
- All younger children (aged two to SK) are encouraged but not required to wear a non-medical mask or face covering while inside the premises.
- Masks are not recommended for children under the age of two.
- Masks should be replaced when they become damp or visibly soiled.
- Children must be supported to wear masks to the greatest extent possible. Resources are available that can be shared with parents/guardians on supporting children in wearing masks. Children and families with masking issues can also be supported by discussing, in consultation with the child's healthcare provider, whether other types of face coverings might work for their child.
- Reasonable exceptions to the requirement to wear masks are expected although should be rare in circumstance (e.g., medical conditions). Requirements and exemptions related to masks must be documented. A letter from the parent or health

provider may be requested. A tip sheet on health and safety practices for children who cannot wear masks can be shared with families.

- Staff members with exceptions to wearing a mask should not be admitted into the premises if they have received their COVID-19 vaccination in the past 48 hours and are experiencing what may be post-vaccine symptoms, as a medical mask must be worn at all times in that circumstance.

Transportation

- All Individuals are required to wear medical masks when providing transportation for children.
- Eye protection for drivers should not interfere with the safe operation of vehicles and is intended to protect drivers during close contact, such as during boarding and exiting.
- The use of non-medical masks or face covering for children grades 1 and above will be required on vehicles. Children below grade 1 should be encouraged to wear masks on transportation.
- Licensees should support accommodations for immunocompromised and otherwise medically vulnerable children, and children with special transportation needs.

Gowns

Gowns must be worn in the following circumstances:

- Cleaning and disinfecting bodily fluids
- While caring for child in isolation

Hand Washing

- Appropriate hand hygiene is one of the most important protective strategies. It is suggested that home care providers and visitors should be trained and able to assist children on appropriate hand hygiene, including the use of alcohol-based hand rub (ABHR), and reinforcing its use.
- It is suggested that ABHR with a minimum 60% alcohol concentration be available (60-90% recommended, ideally at the entry point to each child care room) and/or plain liquid soap in dispensers, sinks and paper towels in dispensers. ABHR is most effective when hands are not visibly soiled. It is recommended that ABHR only be used for children ages two and up, and with adult supervision.
- Proper hand hygiene should be conducted by anyone entering the child care setting and incorporated into the daily schedule at regular intervals during the day, above and beyond what is usually recommended (e.g., before eating food, after using the washroom, etc.)
- Soap and water are preferred as the most effective and safest method for hand hygiene.

Home care providers will perform and promote frequent and proper hand washing.

Staff must wash their hands with soap and warm water in the following situations:

- Before handling food
- Before and after eating
- Before and after diaper check and change
- Before and after toileting
- Before and after contact with bodily fluids

- After handling toxic materials
- Before and after using gloves
- Before and after touching theirs or someone else's face
- When hands are visibly soiled

Children must wash their hands with warm soapy water:

- Before handling food
- Before and after eating
- Before and after toileting
- Before and after wiping their own nose
- When hands are visibly soiled

Food Provision

- Family style meals are permitted to operate provided that food handlers use adequate food handling and safety practices.
- Children must practice proper hand hygiene before self-service of food during family style meals and snacks. Staff must observe children to ensure that they don't pick up items and set them back.
- Where possible, children should practice physical distancing while eating.
- No outdoor food provided by family outside of the regular meal provision except where required and special precautions for handling and service food are in place.
- Children should neither prepare nor provide food that will be shared with others.

Cots and Cribs

Children will have a cot or crib assigned to them. Cots and cribs will be placed to support physical distancing practices.

- Sheets and blankets must be changed between each user
- Cots and cribs must be cleaned and disinfected weekly if designated to one child
- Sheets and blankets must be changed weekly if designated to one child
- Staff must record all these tasks on the Cleaning Schedule for Linens and Cots

Toys and Equipment

Staff must ensure that all toys and equipment are in good repair, clean and sanitary. The agency, home visitor or designate must be advised of any concerns regarding toys and equipment.

- All toys used at the premises must be made of materials that can be cleaned and disinfected easily. Avoid absorbent materials like plush toys.
- Activities should be planned to avoid the sharing of objects or toys.
- If sensory materials (e.g., playdough, water, sand, etc.) are offered, emphasis should be placed on hand hygiene before and after the use of materials.
- Standard cleaning protocols will be followed when a child in care is asymptomatic.

Cleaning and disinfection when a child or staff become ill during care

- When a child or staff becomes ill during care, steps must be taken to clean and disinfect all areas and items that the ill person may have been in contact with as soon as possible.

- The isolation room or area is to be cleaned and disinfected immediately after the child has been sent home.
- All toys and equipment must be immediately removed from the room to be cleaned and disinfected. Any items that cannot be cleaned (e.g., books, cardboard puzzles) should be removed and stored in a sealed container for seven days before they can be put back into rotation.
- If possible, remove children from potentially contaminated area until cleaning and disinfection is complete.

Clean and disinfect frequencies for other surfaces and items:

Cleaning and disinfecting routines **must** be increased as the risk of environmental contamination is higher:

- **Tables and countertops**: used for food preparation and food service must be cleaned and sanitized before and after each use
- Children must not share food, feeding utensils, soothers, bottles, sippy cups. Other tables and chairs being used are to be cleaned and disinfected twice daily and more often as needed.
- **Spills** must be cleaned and disinfected immediately
- **Washrooms and Handwash sinks:** One cohort should access the washroom at a time. Public Health recommendation is that washroom areas must be cleaned in between each use, particularly if different cohorts will be using the same washroom. Washrooms should also be cleaned as often as necessary (e.g., when visibly dirty or contaminated with body fluids).
- **Floors**: cleaning and disinfecting must be performed as required (i.e., when spills occur) and throughout the day when rooms are available (i.e., during outdoor play)
- **High-touch surfaces:** any surfaces at your location that has frequent contact with hands (e.g., light switches, shelving, containers, hand rails, door knobs, sinks, toilets, toilet handles) should be cleaned and disinfected at least twice per day at minimum, but as often as necessary (e.g., when visibly dirty or contaminated with body fluids)
- **Other shared items**: any shared items (e.g., phones, IPADs, IPODs, attendance binders) must be disinfected between users
- All toys must be cleaned and disinfected:
    - weekly when in use by the same cohort.
    - between each cohort's use.
    - daily if in outbreak.
- Mouthed toys must be cleaned and disinfected after every use
- Large equipment and shelving must be cleaned and disinfected every week
- If sensory materials (e.g., playdough, water, sand, etc.) are offered, emphasis should be placed on hand hygiene before and after the use of materials.
- Dramatic play clothes must be laundered after each use
- Floors must be kept clean and dry throughout the day. They must be swept and mopped as necessary by staff.
- Children's cubbies are to be kept neat and checked weekly by staff.
- If outdoor toys and equipment is accessed (e.g., balls, ride-on toys), it is encouraged that items are cleaned and disinfected on a regular basis and when visibly soiled.

- Outdoor play structures (e.g., slides, climbers) do not require cleaning in between cohort use; instead, the focus should be on exceptional hand hygiene before and after outdoor play.
- The Cleaning Schedule for Toys and Equipment must be posted in playrooms.

Daily Cleaning

The Ministry of Health's process for cleaning surfaces will be followed by all care givers.

Cleaners will thoroughly clean and disinfect bathrooms, sweep, and mop floors, spot clean glass, vacuum carpets, dust furniture and shelves, empty garbage and disinfect containers daily.

Coat Room Spaces

1. Use alternate hooks (so that children's clothes and objects do not touch each other).

Activities

1. It is ok to explain the current situation to children, without unduly worrying them, in a language understandable to them.
2. Keep at least a 2-meter distance between children when conducting activities.
3. Reduce the number children gathering in any learning area.
4. Move children amongst learning areas while the other areas are disinfected.
5. Avoid activities that require children to share toys or touch each other (holding hands etc.). If playing board games, let only one child move the pieces.
6. Limit group sizes (E.g. washroom – do not line up).
7. Suspend all sensory play (E.g. water table play, sensory food play).
8. Handwashing will be made a new routine and fun-activity which will be conducted at every hour/two-
hour interval, or more frequently.

Interactions with Infants and Toddlers

1. Provider must continue to supervise and hold bottles for infants not yet able to hold their own bottle to reduce the risk of choking.
2. When holding infants and toddlers use blankets or cloths over clothing and change the blankets or cloths between children.
3. Consider removing cribs or placing infants in every other crib and mark the cribs that should not be used to support physical distancing.
4. Recognizing that physical distancing is difficult with small children and infants, suggestions to support physical distancing include:
a) Planning activities that do not involve shared objects or toys; and,
b) When possible, moving activities outside to allow for more space.
5. Children must not share food, feeding utensils, soothers, bottles, sippy cups, etc. Mouthed toys must be removed immediately for cleaning and disinfecting and must not be shared with other children.
6. Label these items with the child's name to discourage accidental sharing.

Ratios

1. Follow the ratios rules and room capacity rules as mandated by Childcare Licensing.

Snack and Mealtimes

1. Follow all existing rules for cleaning and disinfection and hand hygiene before, during and after meals.
2. Keep one chair distance apart at snack and mealtimes when children are eating.
3. Providers must wear gloves when serving food.
4. Use disposable cutlery where possible.
5. There should be no sharing of utensils.
6. All food preparation and portioning, including snacks, must be done in the kitchen.
7. There is no self-serve or sharing of food. Provider serves all food to the children.
8. Limit the number of personal water bottles. Use cups instead and send them to be washed immediately.
Use paper cups if available.
9. Food remains should not be reused but thrown in the garbage right way.
10. Provider must return dirty dishes and cutlery to the sin or disinecting.
12. If available, all dishes and cutlery must be washed in a dishwasher.
13. There should be no food provided by the family/outside of the regular meal provision of the program
(except where required and special precautions for handling and serving the food must be put in place). 14. Children should neither prepare nor provide food that will be shared with others.

Washroom & Diaper Changing Routines

1. Ensure toilet seats and faucets are cleaned and disinfected after each use.
2. Ensure that diaper changing routines are followed thoroughly as per usual.

Closing

Ensure that all areas (including play material areas) that you will be using the next day are cleaned, disinfected and ready to use.

Provision of Special Needs Resources (SNR) Services

1. Childcare Licensing recognizes that children with special needs and their families continue to require additional supports and services in childcare settings.
2. The provision of in-person special needs services in childcare settings should continue where appropriate. Should questions arise in respect of which service providers are permitted to enter the premises, please consult with your local public health unit. Please work with special needs service providers to explore alternative modes of service delivery where in-person delivery is not possible.
3. Maximum capacity rules do not apply to SNR staff (consultants and enhanced staff) on site (i.e., if they are not counted towards staff to child ratios, they are not included in the maximum capacity rules).
4. Where SNR services are provided through external service providers, licensees and home childcare providers should inform all families of this fact, and record attendance for contact tracing purposes.
5. All SNR staff must be screened before entering the childcare setting, as per the protocol in the screening section above.

Cleaning and Disinfection

Increase Frequency of Cleaning and Disinfecting

1. Twice daily - High-touch areas – doorknobs, handles, phones, tablets (iPads), light switches.
2. Daily - Counters, chairs, tables, toilets, and sinks (after each child has sat/used that area).
3. Put away toy for cleaning and disinfecting as soon as it is mouthed.
4. Floors in the home with children are cleaned twice a day. And once a day for floors without children (e.g. kitchen, hallways).
5. bathrooms, and common areas disinfected and cleaned frequently.

Home Cleaning

1. Follow Public Health Requirements OR
2. Unless specified here, double cleaning procedures based on existing agency requirements.

Toys and learning Material

1. Toys and learning material must be cleaned and disinfected at least once per day.
2. Limit the number of learning materials to use. E.g. in Dramatic area reduce the number of cups, or dolls.
3. Remove materials that are difficult to clean – natural materials, dramatic play clothes, stuffed toys.
4. Replace with toys that you can possibly disinfect in a dishwasher.
5. Quarantine toys that you are unable to clean (put it in a plastic bag, label).
6. Toys and equipment should be cleaned and disinfected at a minimum daily
7. Have designated toys and equipment (e.g., balls, loose equipment) for each child. Where toys and equipment are shared, they should be cleaned and disinfected prior to being shared.
8. If sensory materials (e.g., playdough, water, sand, etc.) are offered, they should be provided for single use (i.e. available to the child for the day) and labelled with child's name

Other Areas & Items

1. Coat room – clean and disinfect at the end of each day.
2. COTS – clean and disinfect at the end of each day.
3. SHEETS, covers, blankets – Wash and dry as required.
4. Where applicable, provide a cup of 'Clean' pens for parents to use when signing the Sign-in Sheet upon dropping off their child. Place the used pens in a cup marked 'Used'. 'Used' pens must be disinfected before placing back in the 'Clean' cup. Alternatively, ask parents to use their personal pen.

Use the Cleaning Schedule Form to track your cleaning routines so not to miss anything. As much as possible, cleaning and disinfecting must be carried out with children not in the room, or in a separate room.

Personal Hygiene
Wash hands, every hour, after wiping noses, and after using the toilet (or use hand sanitizer).
Make sure to cough and sneeze into your elbow.
Follow the Agency Handwashing Posting in the absence of Public Health instructions.

Home Visitors & Directors
1. Have only the necessary number of employees on site to meet the needs of the families.

2. Have the same set of providers (including supply / replacement) work with the same group of children as much as possible. The more you reduce additional contact with different people the less the spread of infection.

3. When planning provider rotations:
a) Avoid providers working across several home locations
b) Have one provider or the ull day schedule
c) Make sure the morning home visitor does not meet the afternoon homevisitor. here applicale
d) Cleaning is every Home Providers' responsibility

Monitoring and Communicating
1. Ensure providers are carrying out their enhanced routines regularly and correctly.
2. Ensure you can communicate with local Public Health in a timely manner for any issues that emerge.
3. Do not hesitate to reach out to the local Public Health Department for directions on situations.

Preventive Procedures for Home Visitors and Directors
1. Director/Home Visitor must be in contact as much as possible to ensure compliance with the instructions to parents, children, and providers.
2. Stay in the office as much as possible and make sure to follow up with the team working with children while respecting the principle of social distancing.
3. Be open without giving into panic, remain flexible and benevolent with your work teams.

Preventive Procedures for Suppliers
1. Suppliers, including the catering company, and visitors are not authorized to enter the provider's home.
2. The supplies should be left at the entrance in a box identified for this purpose or in front of the rear door.
3. Inform suppliers that you must apply strict measures to limit the spread of the virus.
4. If you must buy food at the grocery store, encourage Online shopping and delivery. When you receive the delivery, limit handling or movement in the home. Clean the surfaces that have been in contact with items.

PROGRAM POLICIES:	CHILDREN/STAFF WHO BECOME ILL DURING CARE	SUBJECT	COVID-19
MEDICAL AND HEALTH REQUIREMENTS	EXCLUSION COVID-19		

POLICY

To ensure the health and well-being of children and staff within the early learning and child care setting, children and staff who become ill while attending the program will immediately be isolated and required to leave the program. Staff must consistently assess their health and the health of children in care.

PROCEDURES

When to exclude

Communicable Diseases

If COVID-19 or any other infectious disease is suspected, the parent should be contacted immediately via phone, email or text; and the child must be isolated from the other children and supervised until the parent arrives. Staff should ensure that the child is as comfortable as possible. Home care agency and Regional Health departments will be contacted as needed.

Parents should be advised to use the COVID-19 Screening Tool for Children, Staff and Visitors in Regional Licensed Child Care should follow instructions which may include seeking medical advice or getting a lab-based PCR COVID-19 test . Parents should also follow guidance in this document as to when their child may return to care. Suspected cases of COVID-19 do not require Serious Occurrence reporting. Any child who develops symptoms of ill health including one or more symptoms of ill health related to COVID19 must leave the child care program. Symptoms should not be chronic or related to other known causes or conditions.

The Symptoms most commonly associated with COVID-19 in children include:

- Fever and/or chills (temperature of 37.8°C/100°F or greater)
- Cough or barking cough (continuous, more than usual, making a whistling noise when breathing, not related to asthma, post-infectious reactive airways, or other known causes or conditions the child already has)
- Shortness of breath (out of breath, unable to breathe deeply, not related to other known causes or conditions the child already has)
- Decrease of loss of smell or taste (not related to seasonal allergies, neurological disorder, or other known causes or conditions the child already has)
- Nausea, vomiting and/or diarrhea (not related to irritable bowel syndrome, anxiety, menstrual cramps, or other known causes or conditions the child already has)

Note: Nausea, vomiting and/or diarrhea is not included in the COVID-19 symptoms screening list for adults. Tiredness and muscle aches are included in the COVID-19 symptoms screening for adults. Staff and visitors should be screened for these symptoms.

If the child has any symptoms (including fever, cough, difficulty breathing, vomiting, or diarrhea), the child must be isolated immediately from the other children in a separate room, if available, and supervised at all times.

The parent must be notified to arrange for pick-up of the child. Parents should be asked to ensure that the child is picked up within one hour of the time of the call. If the parent is not reached an emergency contact person will be contacted to pick up the child within the hour.

Every effort must be made to keep the child comfortable until someone arrives to take him or her home.

Staff must document the situation in the Daily Communication Log and notify the agency, home visitor or designate immediately to ensure that this information is managed and recorded in the child's file on the Symptoms of Ill Health.

How to exclude

- Supervise the child in a designated room with a hand washing sink or hand sanitizer available. If a separate room is not available, the ill person should be kept at a minimum two meters physical distance from others.
- Notify parents/caregivers of the sick child for pick up.
- Children older than two years of age should wear a medical mask, if tolerated, and be provided with tissues and reminded of hand hygiene, respiratory etiquette (e.g., sneezing in sleeve) and proper disposal of tissues.
- Only one staff should be in the designated room and attempt physical distancing of two meters. Staff should wear a medical mask, goggles or face shield, gown, and gloves. In addition, staff should perform hand hygiene, avoid contact with child's respiratory secretions and attempt to not touch their face with unwashed hands.
- Increase ventilation in the designated exclusion room, if possible (e.g., open windows).

- All items used by the ill child should be cleaned and disinfected. Any items that cannot be cleaned (e.g., books, cardboard puzzles) should be removed and stored in a sealed container for seven days.
- The isolation room or area is to be cleaned and disinfected immediately after the child has been sent home.
- If the child care program is in a shared setting (e.g., school) follow Public Health direction for notification of others using the space.
- Children or staff who have been exposed to a confirmed case of COVID-19 should be excluded from the child care setting and follow directions from the Regional Health Department.
- All confirmed cases must be reported as a serious occurrence to CCLS and public health must be contacted. Public health guidance and direction will be followed thereafter.

Returning from exclusion due to illness

- Staff/children who are being managed by a specific Region Health Department (e.g., confirmed cases of COVID-19, household contacts of cases) should follow instructions from the Region's Health Department to determine when to return to the premises. Confirmation from Health Department will be required.
- A child can return to program based on the guidance provided by Public Health in the *COVID-19 Screening Tool for Children, Staff, and Visitors in Region's Licensed Child Care*. This may include receiving a negative lab-based PCR COVID-19 test result and being symptom-free for 24 or 48 hours, receiving an alternate diagnosis by a healthcare professional or waiting 10 days from the symptom onset and feeling better.

Staff Illness

Any care giver or persons ordinarily resident on the premises who suspects that they have an infectious disease should follow common sense precautions and should not attend the child care program/premises if they are not well, particularly if their symptoms include any outlined in the COVID-19 screening. Providers must pass the COVID-19 daily screen to commence care. (All results will be recorded in the daily log)

Care givers who begin to experience mild headache, fatigue, muscle aches and/or joint pain (with no other COVID-19 symptoms) and has received their COVID-19 vaccination within the past 48 hours may be experiencing post-vaccine symptoms and would not be required to pause their child care program. If a staff member is still experiencing fatigue, or muscle aches after the 48-hour time frame, they would be required to stop their child care program call parents and agency to arrange pick up of children and self isolate while following public health and agency guidelines.

If a staff member becomes ill during care they should, if possible, isolate themselves immediately and arrange pickup for children and call agency in addition to following public health and agency guidelines/direction.

A provider or person ordinarily resident who presents with symptoms of ill health must notify the agency, complete the Provincial online self-assessment tool and follow instructions which may include seeking medical advice or getting a lab-based PCR test. provider must follow the direction of their healthcare provider and/or the Regional Health Department.

Where alternative or rescheduled care due to suspected and or confirmed cases of covid-19 is required; it will be organized once public health advises the agency to resume care.

Pick-Up and Drop-Off

1. Avoid all physical contact with parents – No handshakes hugs etc
2. Monitor children's temperature:
a) On arrival at the home, and any time a child shows symptoms of illness.
b) The Home Visitor/Director/Provider will print the attached Daily Health Check Form to monitor the temperature of all children throughout the day.
c) NOTE: The Provider must use the Record of Illness Form – Individual to record information (symptoms, age, room, etc.) concerning an ill child.
3. Ask parents to drop-off and pick-up their children at staggered intervals (to avoid interaction).
4. Parents should drop-off children at the main entrance and have the provider bring the child into their home. Parents may wait outside the main entrance at pick-up time.
5. Designate an area near the main entrance to conduct screening. The area should allow for physical distancing (minimum 2 meters) between provider and the person being screened. If physical distancing cannot be maintained, providers conducting screening should wear appropriate personal protective equipment, such as mask and gloves. Alcohol based hand rub (ABHR) containing at least 60-90% alcohol content should be available at the screening station for hand hygiene.
6. Actively screen parents and children, prior to entry/drop-off for respiratory symptoms. A central record of monitoring should be kept. Examples of screening questions include, but not limited to the following:
a) Do you or the child or any member of your household have any of the following symptoms:
Enhanced Preventive Procedures for Providers (Up to date screening sheet posted near entrance)
- Fever
- New or existing cough
- Difficulty breathing
- Sore throat
- Vomiting and/or diarrhea?

b) Have you or the child had close contact in the last 14 days with a person with acute respiratory illness?
c) Have you or the child had close contact (within 2 meters) in the last 14 days with someone that is a confirmed/ probable case of COVID-19 (including those who are waiting for COVID testing result)?
d) Have you or the child traveled outside of Canada, within the last 14 days?
7. Anyone who answers YES to any of the question should not be permitted to enter the provider's home.
8. Make ABHR available at the screening stations for people who have answered NO to all questions for use prior to entry into the home. When ABHR is used on children they must be supervised. To ensure proper use, follow the manufacturer's directions (consent shall be given).
9. Thermometers must be covered with single-use protective covers (which is disposed after each use) or cleaned and disinfected with an outbreak level disinfectant before reuse.
10. Post signage in visible areas which clearly explains the screening process and the rules and conditions for entry.
11. Handwashing for children will be done immediately upon arrival.

Outdoor Time

1. Stagger outdoor time to small groups of children
2. Avoid usage of balls and other toys that require frequent sharing.
3. Disinfect riding toys between uses.
4. Where the outdoor play area is large enough to accommodate multiple providers may divide the space with physical markers to ensure children remain separated by at least 2 metres.
5. If play structures are to be used by more than one child, the structures can only be used by one child at a time and should be cleaned and disinfected before and after each use by each child.
6. Where there are challenges securing outdoor play space, we can have alternate outdoor arrangements (e.g. community walk). Follow physical distancing practices when possible.

Nap & Rest Time

1. If possible, maintain a distance o one cot apart spacing when children sleep.
2. If possible, in the infant sleep area, maintain a distance with alternative cribs spacing when infants sleep.
3. Each infant's bedding must be used for him/her only. All bedding must be laundered when wet or soiled, additionally: I Provider launders eddings

a) Wet or soiled bedding must be placed in a suitable container until it can be laundered.
b) Open laundry machine door first before touching dirty clothes to transfer.
c) Cots, cribs, and mattresses must be wiped daily with an appropriate disinfectant, and when wet or soiled.

4. Wash hands after clearing up the sleep area before returning to the children.

Maintain Open Communications – Provider & Parents

Communication is Essential

1. Maintain open communication with your e d re. Share all updates including status and emergency procedures.
2. Providers at home, keep informed via email and telephone calls.
3. Post a description of the symptoms associated with COVID-19 at the entrance and inside the home and

ask parents to seek professional medical assistance if they are concerned about their child:

a) Canada: https://www.canada.ca/en/public-health/services/diseases/2019-novel-coronavirus-infection/symptoms.html#s.
b) US: https://www.cdc.gov/coronavirus/2019-ncov/prepare/children.html.

4. Emergency contact: refer to the Emergency Information Posting (EIP).
5. Inform parents of all changes to procedures so they are aware and are following them.

PROGRAM POLICIES SUBJECT COVID-19

DOCUMENTATION
Covid-19 Outbreaks

COVID-19 Outbreaks

Policy

A COVID-19 outbreak may be declared by public health when, within a 14-day period, there are two or more confirmed COVID-19 cases in children, staff or visitors with an epidemiological link (e.g., cases in the same home, cases that are part of the same before and after school care cohort), where at least one case could have reasonably been acquired within the premises.

Public health will help to determine whether epidemiological links exist between cases and whether transmission may have occurred in the child care setting.

Procedures

If an outbreak is declared, public health will devise a plan to manage the outbreak. This includes:

- Determining whether to close particular child care rooms, cohorts or full closure of the home premises.
- Determining if children need to be sent home and providers/family members need to self isolate.
- Determining who to contact and inform of the outbreak.

As per the Serious Occurrences policy, a report must be filed to the Ministry of Education for any confirmed COVID-19 cases of a child, staff or student.

Parents must be notified by means of a phone call or email, text and an appropriate information sheet that is posted on the entrance door unless otherwise notified by Public Health. Notification and information may be handed out to parents upon request.

Infection Control
Staff must refer to Outbreak Control in their Regional Health Department guidelines.

•Increase washing/sanitizing and surface cleaning/disinfection throughout, regardless of scheduled cleaning.

•Anyone experiencing symptoms of illness should be excluded from the premises. This includes alternate staff, parents, children, volunteers and students. Family members must isolate as per directions of Health Department

Surveillance and Monitoring
Information should be documented on data tracking form.

PROGRAM POLICIES:

SUBJECT COVID-19

PROFESSIONAL REQUIREMENTS AND LIABILITIES

SUPERVISION OF CHILDREN DURING COVID-19

POLICY

To ensure all staff are aware of the expectations regarding the supervision of children.

The Child Care and Early Years Act (2014) identifies the following references for supervision of children:

Supervision by an Adult

Every licensee shall ensure that every child who receives child care at a child care centre it operates, or at a premise where it oversees the provision of home child care, is supervised by an adult at all times, whether the child is on or off the premises.

Intent:

This provision protects the safety and well-being of children, by requiring that they be supervised by an adult at all times while receiving child care.

Group Sizes

Effective September 1, 2020, cohort sizes reflect that maximum group sizes set out under the Child Care and Early Years Act, 2014 (CCEYA). Staff and students are not included in group sizes but should be assigned to a specific group where possible. Children are permitted to attend on a part-time basis and as with children attending full time, should be included in one group and should not mix with other groups.

Movement of supervisors or designates, staff and students on educational placement between child care locations and between licensed age groups is permitted. In the event of an outbreak, all movement of supervisors or designates, staff and students should cease until the outbreak is over. Reducing the movement of staff and placement students where possible is encouraged to minimize potential for transmission.

Ratios set out under the CCEYA must be maintained. Reduced ratios are permitted as set out under the CCEYA, provided that groups are not mixed. Reduced ratios are not permitted at any time for infants.

Maximum capacity rules do not apply to Special Needs Resource staff on site (i.e. if they are not counted towards staff to child ratios, they are not included in the maximum capacity rules).

Mixed age grouping is permitted as set out under the CCEYA where a director approval has been granted on the licence.

While on duty, staff are responsible for the safety of the children at all times and constant supervision is required. Staff are assigned responsibility for a specific group of children. Staff must determine each child's developmental ability (using developmental checklist materials) in order to plan and implement a program that effectively meets the child's individual needs. Planning the program with consideration given to the children's abilities as well as general safety practices is an essential component of effective supervision of children.

Space Set-Up and Physical Distancing

- Cohorts must have their own assigned indoor space separated from other cohorts by a physical barrier. The physical barrier needs to be a minimum of 8 feet and as wide as the room will allow.
- When in common spaces a physical distancing of at least 2 metres must be maintained between different groups.
- Low contact activities are permitted indoors. For children in grade 1 and up, masking is encouraged but not required if a minimum of two metres distance can be maintained between groups and as much as possible within a group.
- High contact physical activities should take place in outdoor settings only. Masking is not required outdoors for high contact physical activities.
- Singing is permitted indoors; masking is encouraged but not required for singing indoors if a minimum of two metres distance can be maintained between cohorts and as much distancing as possible maintained within a cohort.
- In shared outdoor space, groups must maintain a distance of at least 2 metres between groups and any other individuals outside of the cohort.

The risk associated with transmission with shared objects is low. Instead of regular cleaning of shared objects, the focus should be on regular hand hygiene and respiratory etiquette to reduce the risk of infection related to shared equipment.

Large playground equipment does not require disinfection.

PROCEDURES

- Constant visual supervision of children is essential. All staff are responsible for ensuring that all exit points are monitored, indoors and outdoors.
- Communication is an essential component of working as a team. Staff must ensure that they have informed each other if they are leaving the room with children or arriving with additional children.

- When one member of the team leaves the area, the team partner(s) is/are responsible for all children within the area.
- Children's attendance must be recorded immediately upon arrival or departure from the program. Attendance must also be checked during routine changes of the day. This ensures a matching of the attendance record with the children in care.
- Staff must ensure that children are safely released to their parent or guardian. As children tend to have regular hours of care, it is important for staff to be aware of when to expect a child's arrival or departure.
- In order to supervise an area effectively, staff must position themselves to achieve visual supervision of the children, indoors and out. If a child needs to leave the group for any reason, staff must ensure that an adult adequately supervises him or her.
- When setting up a playroom, staff must ensure that consideration is given to the types of activities planned and the physical set-up of the room. Staff must be able to effectively monitor the activities to maintain a safe environment for the children.
- In shared outdoor space, mixing between groups and any other individuals outside of the group is permitted, though physical distancing should be encouraged between groups as much as possible.
- Staff are required to count the number of children in their care before and after the movement of children from within the premises, when outside in the playground and when moving children to and from the playground, and at any time that the provider takes the children off the premises.
- Staff must ensure that children are aware of the rules of the playroom and playground. This will help children understand the expectations for safe play.
- Staff must recognize the need for extra precautions regarding the supervision of children when there is a change to regular day-to-day programming. For example, when supply staff are filling in, it is necessary to take additional precautions.
- Staff must be constantly aware of what is happening around them. By listening carefully and monitoring the room a potential conflict or safety hazard may be prevented.
- Staff should continue to supervise and hold an infant's bottle if the infant is not yet able to hold their own.
- It is the expectation that staff regularly review and keep up to date in their knowledge of appropriate policies and procedures.

Attendance Records

In addition to attendance records for all children receiving child care, all child care licensees are responsible for maintaining daily records of anyone entering the child care facility/home.

These records must include all individuals who enter the premises (e.g., parents and guardians dropping off children, cleaners, people doing maintenance work, people providing supports for children with special needs, those delivering food).

• Records are to be kept on the premises (home) and along with name and contact information must include an approximate time of arrival and time of departure, and screening completion for each individual. (Recorded In Daily Log Book or Separate notebook/document) (These Records are kept for 1 year on the premises).

• Records must be kept up-to-date and available to facilitate contact tracing in the event of a confirmed COVID-19 case or outbreak (i.e., records can be made available to public health within 24 hours of a confirmed COVID-19 case or outbreak)